

Session Note - March 20, 2026

Patient: Rachel Morrison

Therapist: Dr. Karen Abbott, LCSW

Presenting Concerns:

Patient presents with significant distress today. Reports thinking about suicide daily for the past two weeks. Has developed a specific plan to overdose on prescription medications she has been stockpiling. States she has written goodbye letters to her children. Denies immediate intent but admits the urge is getting stronger. Severity is high. Triggers include custody hearing next week and ongoing conflict with ex-spouse.

Session Summary:

Emergency-focused session. Conducted thorough risk assessment. Safety plan reviewed and updated with patient. Discussed lethal means restriction - patient agreed to have her sister secure all prescription medications from the home tonight. Emergency contacts notified with patient consent, including sister Maria Morrison and ex-husband (for child safety awareness). Reviewed crisis hotline numbers and local emergency resources. Patient contracted for safety through next session.

Mood Assessment:

Current mood: 2/10 (hopeless, overwhelmed)

Mood change from last session: Significant decline

Energy level: Low

Emotional themes: hopelessness, despair, guilt about impact on children

Affect observed: Tearful, constricted

Affect congruence: Congruent with reported mood

Mental Status Examination:

Appearance: Disheveled, appeared fatigued

Behavior: Cooperative but emotionally labile

Speech: Slow rate, soft volume, occasional pauses

Thought process: Linear but preoccupied with suicidal thoughts

Thought content: Active suicidal ideation with specific plan (overdose), stockpiled means

Perception: No hallucinations or delusions

Cognition: Intact, alert and oriented

Insight: Fair - recognizes need for help but feels overwhelmed

Judgment: Impaired by emotional state

Risk Assessment:

Suicidal ideation: Active, with specific plan (medication overdose). Frequency: daily for two weeks. Has stockpiled prescription medications. Written goodbye letters to children. Denies immediate intent but urge intensifying. Protective factors: children, therapeutic relationship, agreed to safety plan.

Self-harm: None reported.

Homicidal ideation: None.

Risk level: High.

Safety plan: Updated - remove access to medications, crisis contacts identified, agreed to call 988 if urges intensify.

Interventions Used:

- Crisis intervention and safety planning
- Lethal means restriction counseling
- Supportive therapy
- Emergency contact notification

Techniques effective for session - patient agreed to safety measures.

Skills reinforced: Crisis line usage, distress tolerance skills

Homework assigned: Give all medications to sister tonight, call therapist if urges worsen

Previous homework: Not discussed due to crisis focus

Diagnoses:

Primary diagnosis: Major Depressive Disorder, recurrent, severe

Primary diagnosis code: F33.2

Diagnosis status: Worsened

Treatment Progress:

Overall progress: Significant regression due to custody stressor

Treatment phase: Crisis

Barriers: Acute stressor (custody hearing), access to means, social isolation

Strengths: Therapeutic alliance, willingness to engage in safety planning

Treatment goals: Immediate safety stabilization, reduce suicidal ideation

Plan:

- Increase session frequency to twice weekly until crisis stabilizes
- Next session: March 22, 2026
- Verify lethal means restriction completed
- Consider psychiatric hospitalization if safety plan adherence falters
- Coordinate with psychiatrist Dr. Patel for medication adjustment
- Level of care: Outpatient with intensive monitoring, consider inpatient if needed